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Bwrdd Iechyd Prifysgol Betsi Cadwaladr University Health Board

# ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT DIRECTORATE

## OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

**ANAESTHETIC REFERRAL & ASSESSMENT OF THE MORBIDLY OBESE (BMI >40) PARTURIENT**

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## 1. Introduction and Purpose

It is important that patients with morbid obesity are referred to the anaesthetist in advance of delivery so that they can be adequately assessed. This is with particular regard to assessing their airway. In addition, advice can be given regarding regional anaesthesia for their delivery.

## 2. Scope

This guideline is intended as a reference for obstetricians & anaesthetists.

## 3. Points to consider

- All morbidly obese patients with a BMI > 40 should be referred by the obstetric team for an antenatal anaesthetic assessment before 36 weeks gestation.

- A Consultant Obstetric Anaesthetist will then arrange an appointment to assess the patient in the antenatal anaesthetic clinic.

- The patient will in addition be reviewed by the labour ward duty anaesthetist on arrival in labour or alternatively assessed on the day of their elective Caesarean Section by the anaesthetist undertaking the list.

- On arrival of the patient on labour ward the duty obstetric anaesthetist (551 pager) should be paged by the midwife to come and assess the patient and discuss early epidural analgesia, and in particular its benefits in morbid obesity. If the patient's airway is of concern then he/she should contact the on call consultant anaesthetist.
  If subsequently the patient needs an emergency Caesarean then the duty obstetric anaesthetist should notify the on call consultant anaesthetist in all cases where the BMI exceeds 40.

## 4. References

1. Lewis,G (ed) 2007. Confidential Enquiry into Maternal and Child Health (CEMACH). Saving Mothers' Lives: reviewing maternal deaths to make motherhood safer- 2003-2005.